جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : نادية محمد فؤاد محمد

المحافظة : القاهره **الحي / المدينة :** عين شمس **العنوان :** 13ش المعادي الجديدة البساتين - القاهرة

رقم البطاقة : 27307180102949 **رقم التليفون :** 01222150082 **تاريخ تسجيل الإستمارة :** 17-10-2026

العمر(تاريخ الميلاد) : 18-07-1973 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(----------)

المقر بما فيه
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة الاجابات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نادية محمد فؤاد محمد
National ID: 27307180102949

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشان تقرير طبي للمريض
بقسم:	رقم طبي:

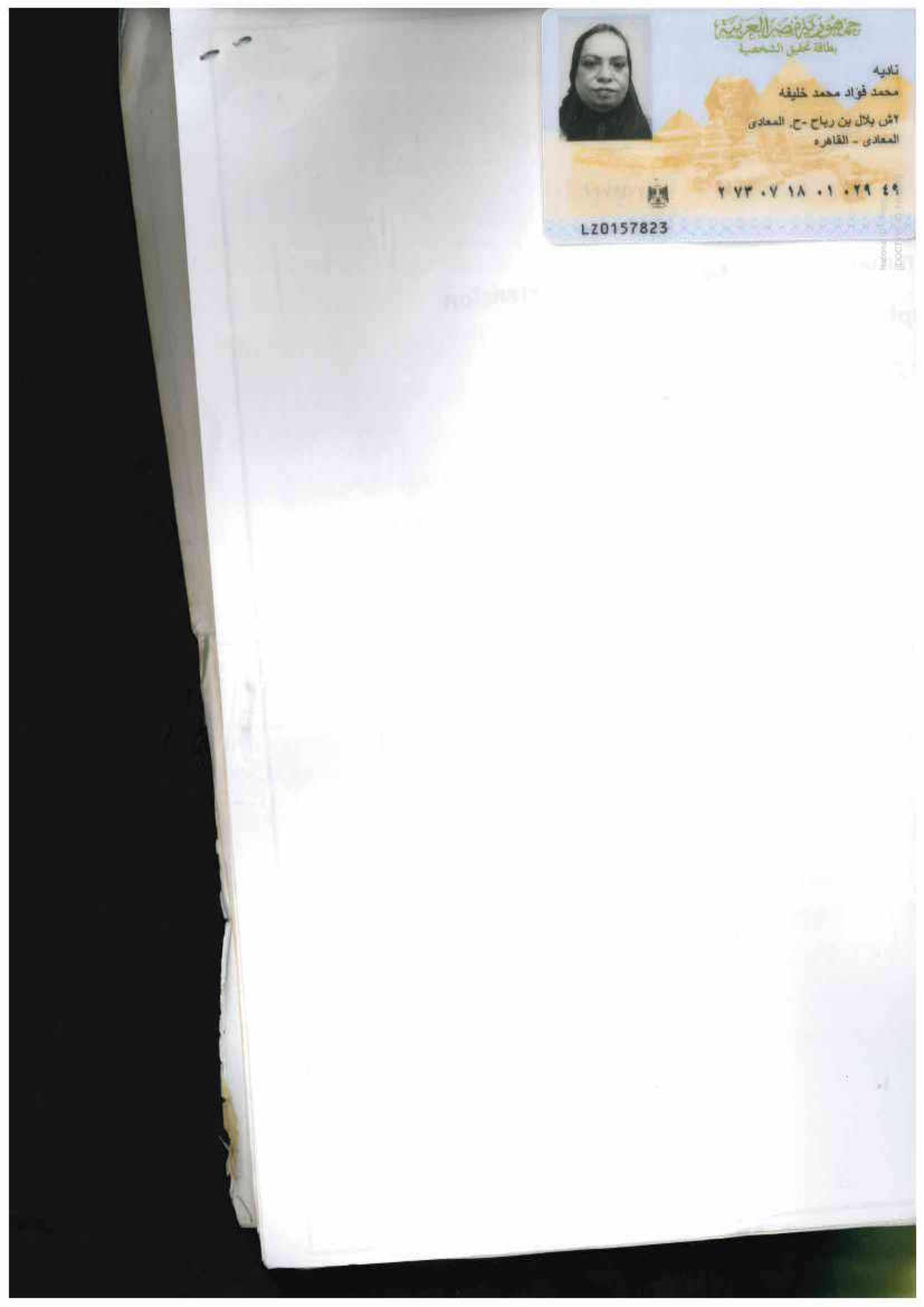
نتشرف بأن نوضح فيما يلي تقريرا عن حالته

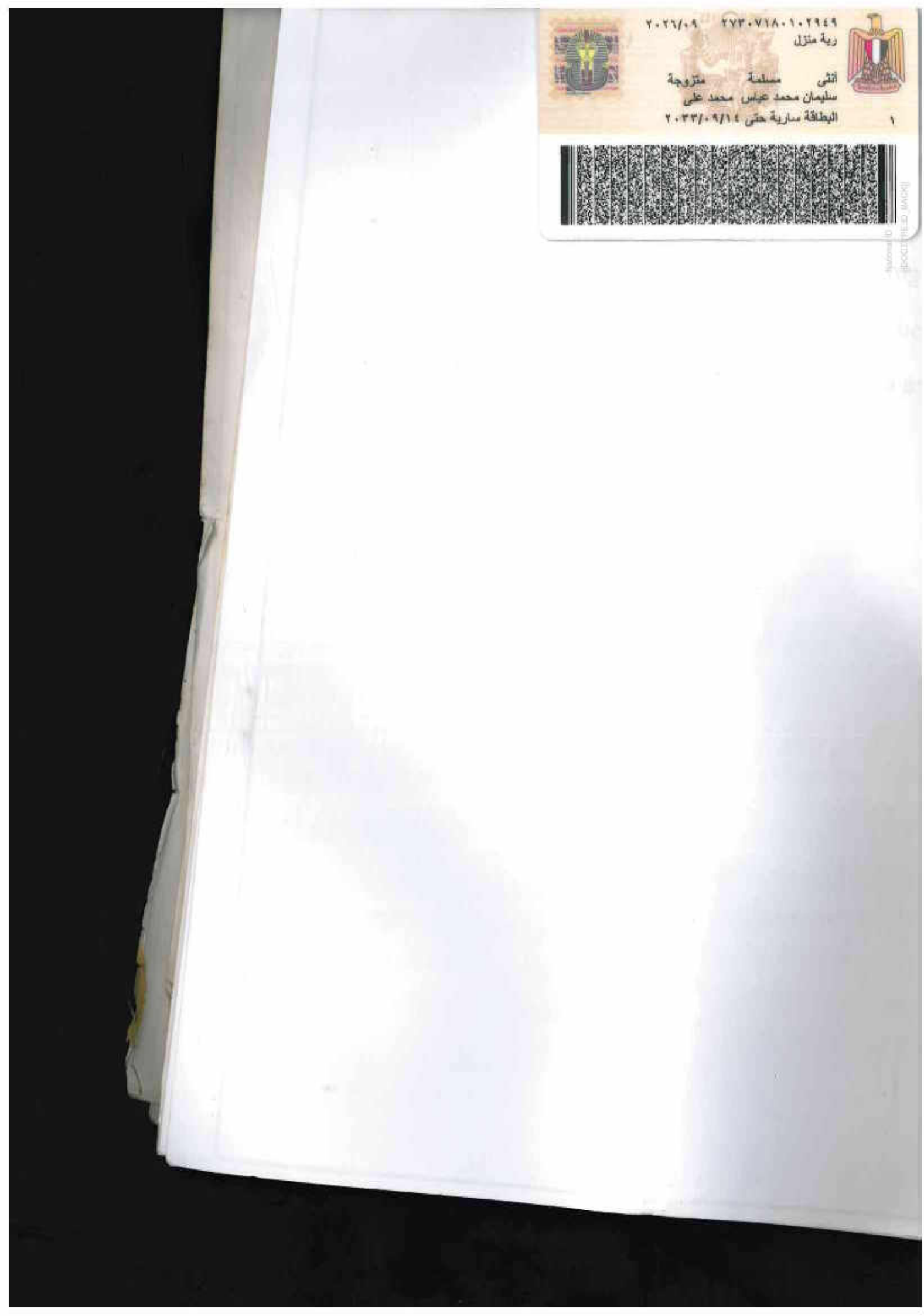
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bilateral mammogram and breast ultrasound

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
ناديه
محمد فؤاد محمد خليفه
٢ش بلال بن رباح -ح. المعادى
المعادى - القاهره
٢٧٣٠٧١٨٠١٠٢٩٤٩
LZ0157823


٢٧٣٠٧١٨٠١٠٢٩٤٩ ٢٠٢٦/٠٩
ربة منزل
أنثى مسلمة متزوجة
سليمان محمد عباس محمد على
البطاقة سارية حتى ٢٠٣٣/٠٩/١٤
١

مركز اورام دار السلام
قسم المعمل

Pathology Report

Name : ناديه محمد فؤاد

7-6-2018.

Age#44years

Clinical data: Left breast mass

Specimen Type: Breast tissue specimen and axillary lymph nodes

Date : 2-7-2018 File Number: 728

Gross Description:

Two specimens were received designated as 1&2

1 – A piece of mammary tissue 7x7 cm with partial skin covering 3x1 cm . Serial sections revealed a firm grayish white mass 2.8x 2 cm located 1 cm under the skin and 1.5 cm away from deep margin

2 - Fibrofatty tissue piece entangling multiple nodules up to 1x1 cm

Microscopic Examination:

Sections examined revealed a case of invasive duct carcinoma G 2 with desmoplastic stroma and mild lymphocytic infiltrate. Peritumeral lymphovascular invasion was present . Associated DCIS (Comedo,cribriform , clining) low to intermediate grade , constituting about 10 % of tumour area the extending margin were free of tumour extension . Axillary lymph nodes showed malignant deposits in 6 out of 11 nodes (6/11) with capsular invasion

Diagnosis:

Left breast mass

Invasive Duct Carcinoma G 2

T2N2Mx

All resection margins were free of tumour extension

Axillary lymph nodes status (6/11)

ER , PR , Her 2 are available on request

PROF. Dr.Mona Sakr
د.منى صقر

Dr. Inas Mohamed Ali.

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مركز اورام دار السلام
" هرمل سابقا "

مركز أورام
دار السلام
هرمل سابقا

تقرير طبي
Medical Report

السيد / .....

تحية طيبة وبعد ،،

نتشرف بأن نوضح فيما يلي تقريراً عن حالة المريض : نادية محمد فؤاد

بقسم : .....

تاريخ وساعة الدخول : / / ٢٠١

تاريخ وساعة الخروج : / / ٢٠١

وبعد الكشف والفحوصات والمتابعة وجد أنه :

التشخيص :
Left Breast abscess
after Surgery MRM

العلاج :
Drainage of the abscess

التوصيات :

تحريرا في ٢٠١ /١٢/٣

الطيب المعالج
د/ آلاء عبد العزيز

يعتمد ،
مدير المركز

مطبعة وزارة الصحة - ف

مركز أورام دار السلام
هرمل سابقا

Patient name: نادية محمد فؤاد

Date: 16/11/2020

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had left CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- No right breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Left breast LOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound also showed left breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- no right breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left infra and supra clavicular regions.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the right breast (BIRADS 1)

Yours truly

Dr\ Walaa Abdel _aziz (FEBR, MSc)

Dar El-Salam Caner

Cente

Radiology Department

أورام دار السلام

مركز أورام دار السلام

الأشعة

Name: Mrs. Nadia mohamed
Date : 25 September 2018

Soft tissue ultrasound examination of left breast revealed: -

- Post conservative surgery of the left breast for breast mass follow up revealed: a well-defined hypoechoic oval mass is seen at 1 – 3 o'clock , it shows internal echogenic septa with no internal vascularity seen at CCDI related to operative bed .
- Subcutaneous edema is seen related to the skin incision .
- Predominantly fibro-glandular breast parenchyma with no areas of architectural distortion.
- No suspicious masses could be detected.
- No suspicious clusters of microcalcifications could be detected.
- Intact skin and subcutaneous tissues.
- No evidence of pathological axillary lymphadenopathy .

OPINION:

- picture is suggestive of post-operative subcutaneous collection and subcutaneous edema for follow up and further assessment if clinically quarantined .

Best regards

Dr/ Maataz Adel .

ass

Carcinoma G 2

margins were free of tumour extension

h nodes status (6/11)

2 are available on request

PROF. Dr.Mona Sakr

د. من صقر

Dr. Inas Mohamed Ali.

مركز أورام دار السلام
قسم المعمل

Pathology Report

Name : ناديه محمد فؤاد

Age#4years

7-6-2018.

Clinical data: Left breast mass

Specimen Type: Breast tissue specimen and axillary lymph nodes

Date : 2-7-2018 File Number: 728

Gross Description:

Two specimens were received designated as 1&2

1 – A piece of mammary tissue 7x7 cm with partial skin covering 3x1 cm . Serial sections revealed a firm grayish white mass 2.8x 2 cm located 1 cm under the skin and 1.5 cm away from deep margin

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Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m3489

اسم المريض	: نادية محمد فؤاد	النوع	: Female
السـن	: 45 Y	العنوان	: السلام
تاريخ اخذ العينة	:	تاريخ استلام العينة	: 04/07/2018 12:21
طالب التحليل	: مركز اورام دار السلام	تاريخ التقرير	: 17/07/2018 12:02
عناية	: مركز اورام دار السلام		

Primary Sample :
Paraffin Block

Test :
ER &PR & HER2 neu (ER&RP : Harvey scoring system - Her2neu : CAP scoring system)

Comment :

Technique :
sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (IE2)Rabbit monoclonal primary antibody.
Sample recieved was treated by : -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.

Result :
ER:moderate nuclear staining of 90 % of tumour cells
PR: strong nuclear staining of 90 % of tumour cells
HER2/neu protein over expression :discontinuous membrane staining of tumor cells.

Referal Lab :

Comment :

Conclusion :
ER: POSITIVE (SCORE 7/8)
PR: POSITIVE (SCORE 8 /8)
HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE (SCORE 1+)

Pathologist: د/أيمان رمزى

Lab Director: د/ تريزا ماهر

9 El-sheikh Rehan Street, Abeideen, Cairo
el.(165)7948544 -7950096 Fax. (202)7948544
obile . 01144297292
ebsite:http://www.healthlabs.net
ail:pathologychl@yahoo.com
ebook: قسم الباثولوجى - الادارة المركزية للمعامل

This Test Is Not Accredited By EGAC Nor Recognized By ILA

يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها

DAR EL_SALAM HOSPITAL

LABORATORY DEPARTMENT

Sample ID	out9999888	Patient ID	
Name	nadea moh foad		
Category	Female	Collection Date	26-Dec-2018
Age	-	Reg. Date	26-Dec-2018
Ref. Dr		Analyst	
Sample Type	SERUM	Location	
Sample Remark			
Patient Remark			

Sr.No.	Test	Result	Flag	Normal Range (Age/Category not defined.)
1	ALT (GPT)	24.0 U/L		0.0 - 31.0 U/L
2	AST (GOT)	24.6 U/L		0.0 - 31.0 U/L
3	Creatinine	0.97 mg/dl		0.60 - 1.10 mg/dl
4	Total Bilirubin	0.42 mg/dl		0.10 - 1.20 mg/dl
5	Urea	23.7 mg/dl		17.0 - 43.0 mg/dl

Completion Date 26-Dec-2018 12:38

Note : Tests have been performed on fully automated analyzer:- respons 920

Print Date 26-Dec-2018 12:38

Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m3489

اسم المريض	: نادية محمد فؤاد	النوع	: Female
السن	: 45 Y	العنوان	: السلام
تاريخ اخذ العينة	:	تاريخ استلام العينة	: 04/07/2018 12:21
طالب التحليل	: مركز اورام دار السلام	تاريخ التقرير	: 17/07/2018 12:02
العناية	: مركز اورام دار السلام		

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Referal Lab :

Comment :

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PR: POSITIVE (SCORE 8 /8)
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يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها

Fatth Islamic Center
C. T. - M.R.I.
U.S. - Duppler - E.C.G. Stress
E.E.G. - E.M.G. - Mamm. - Eye Unit

الجمعية الشرعية
مركز الفتح الإسلامى
أشعة مقطعية بالكمبيوتر - الرنين المغناطيسى
موجات صوتية - دوبلر ملون - رسم قلب بالمجهود -
رسم عضلات - فحص ثدى - وحدة عيون

NAME : MRS. DALIA MOHAMED FOUAD
DATE :05-05-2018

Bilateral Soft Tissue Mammography With Complementary Ultrasound Reveals

- *Both breasts display heterogeneously dense fibroglandular parenchyma (ACR C)*
- ***Left breast shows LOQ irregular shaped dense lesion with speculated margins with overlying microcalcifications and surrounding architecture distortion, complementary US shows an irregular shaped hypo echoic lesion, located at 4 o'clock measures 22 x 17 x 16 mm with surrounding desmoblastic reaction, posterior shadowing and internal vascularity in color Doppler US***
- *Bilateral prominent retroareolar ducts with clear contents. No right breast solid or cystic lesions.*
- *Normal skin thickness and contour*
- *Left axilla shows few enlarged LNs, 2 of them show lost hilum, largest measures 15 x 20 mm, RT likely non specific axillary lymph nodes of preserved fatty hilae.*
- *No pathologically enlarged LT infra or supraclavicular LNs.*

CONCLUSION:

- ***Left LOQ malignant looking lesion with infiltrated left axillary LNs as described for histopathology correlation, (BI-RADS V).***

MUCH OBLIGED YOURS,
Dr. Sara Abd Elghaffar, M.Sc, FEBR

CATEGORY		Management	Likelihood of cancer
0		Needs further imaging study	
1	Negative	Routine screening	
2	Benign finding	Routine screening	Essentially 0%
3	Probably Benign finding	Short term follow up	Essentially 0%
4	Suspicious abnormality	Tissue diagnosis	(>0% but <2%) 4a low suspicion for malignancy(>2% to <10%) 4b moderate suspicion for malignancy(>10% to <50%) 4c high suspicion for malignancy (>50% to <95%
5	Highly suggestive of malignancy	Tissue diagnosis	~95%
6	Proven malignancy by biopsy	Surgical excision when clinical appropriate	

مركز أورام
دار السلام
مصر سابقا

ز اورام دار السلام
سم المعمل

Pathology Report

Name: ناديه محمد فؤاد

Age: 45years

Clinical data: Left breast suspicious lesion.

Specimen Type: FNAc from left axillary lymph node.

Date : 15/5/2018 **File Number:** 728/2018

Gross Description:

2 mls of reddish fluid received in a syringe for cytology.

Microscopic Examination:

The examined smear showed:

Moderate cellular smear showing clusters of malignant epithelium
ells exhibiting high N/C ratio . The background is haemorrhagic with a
ew mature lymphocyts. Complete lymph node excision biopsy is
nandatory for confirmation and for IPT (ER,PR,Her2)

iagnosis:

NAc from left axillary lymph node

ositive for malignant cells for confirmatory complete lymph node
cision biopsy .

مركز اورام دار السلام
قسم المعمن

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Diagnosis:

Left breast mass

Invasive Duct Carcinoma G 2

T2N2Mx

All resection margins were free of tumour extension

Axillary lymph nodes status (6/11)

ER , PR , Her 2 are available on request

PROF. Dr.Mona Sakr

د. منى صقر

Dr. Inas Mohamed Ali.

وزارة الصحة
أمانة المراكز الطبية المتخصصة
مستشفى معهد ناصر
Nasser Institute Hospital
معهد ناصر
مركز الأورام
وحدة الطب النووي

NASSER INSTITUTE
ONCOLOGY CENTER
NUCLEAR MEDICINE UNIT

Oncology Center مركز الأورام
خطابات أورام

Patient Name: Nadia Mahmoud Foaad. ***Date: 14/7/2018***

Tc-99m MDP bone scan

Technique :Delayed Whole body bone scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Delayed Static Phase:-

- Degenerative changes are noticed in dorso-lumbar vertebrae..
- Arthritic changes at knees
- Both kidneys are normally visualized.

Opinion: -

- ***No definite evidence of metastatic osseous deposits.***
- ***Follow up bone scan is recommended.***

Thanks for reference

Nuclear medicine Unit
Huda Fathy,.MD
Walid Omar, M.D.

١٣٥١ كورنيش النيل - أغاخان - شبرا - القاهرة - ت : ٢٢٠٣٩١٦٤ - ٢٢٠٣٩٣٥٦ فاكس : ٢٢٠٣٩٢٣٦
1351Korniesh El Nile - Aghakhan - Shoubra - Cairo Tel : 22039164 - 22039356 Fax : 22039 236

Bone Scan
[[DOCTYPE:BONE SCAN]]

Fatth Islamic Cente,
C. T. - M.R.I.
U.S. - Duppler - E.C.G. Stress
E.E.G. - E.M.G. - Mamm. - Eye Unit

الجمعية الشرعية
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رسم مخ - رسم عضلات - فحص ثدى - وحدة عيون

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5	Highly suggestive of malignancy	Tissue diagnosis	>95%
6	Proven malignancy by biopsy	Surgical excision when clinical appropriate	

٥٢ شارع ١٠ بالمعادى ت & ف ٢٣٥٨٧٢٣٢ - ٢٣٥٨٥٨١٩ - ٠١١٠٠١٥١٣٨٠ - ٠١٠٢٨٤٨٣٦٩٥

مركز اورام دار السلام
قسم المعمل

Pathology Report

Name: ناديه محمد فؤاد

Age: 45years

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Diagnosis:

FNAc from left axillary lymph node

Positive for malignant cells for confirmatory complete lymph node excision biopsy .

PROF. Dr. Mona Sakr .

د. منى صقر

Dr. Inas Mohamed Ali.

Fatth Islamic Center
C. T. - M.R.I.
U.S. - Duppler - E.C.G. Stress
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الجمعية الشرعية
مركز الفتح الإسلامى
أشعة مقطعية بالكمبيوتر - الرنين المغناطيسى
موجات صوتية - دوبلر ملون - رسم قلب بالمجهود -
رسم مخ - رسم عضلات - فحص ثدى - وحدة عيون

NAME : MRS. DALIA MOHAMED FOUAD
DATE :05 -05-2018

Bilateral Soft Tissue Mammography With Complementary Ultrasound Reveals

- *Both breasts display heterogeneously dense fibroglandular parenchyma (ACR C)*
- ***Left breast shows LOQ irregular shaped dense lesion with speculated margins with overlying microcalcifications and surrounding architecture distortion, complementary US shows an irregular shaped hypo echoic lesion, located at 4 o'clock measures 22 x 17 x 16 mm with surrounding desmoblastic reaction, posterior shadowing and internal vascularity in color Doppler US***
- *Bilateral prominent retroareolar ducts with clear contents. No right breast solid or cystic lesions.*
- *Normal skin thickness and contour*
- *Left axilla shows few enlarged LNs, 2 of them show lost hilum, largest measures 15 x 20 mm, RT likely non specific axillary lymph nodes of preserved fatty hilae.*
- *No pathologically enlarged LT infra or supraclavicular LNs.*

CONCLUSION:

- ***Left LOQ malignant looking lesion with infiltrated left axillary LNs as described for histopathology correlation, (BI-RADS V).***

MUCH OBLIGED YOURS,

Dr. Sara Abd Elghaffar, M.Sc, FEBR

CATEGORY		Management	Likelihood of cancer
0		Needs further imaging study	
1	Negative	Routine screening	Essentially 0%
2	Benign finding	Routine screening	Essentially 0%
3	Probably Benign finding	Short term follow up	(>0% but <2%)
4	Suspicious abnormality	Tissue diagnosis	4a low suspicion for malignancy(>2% to <10%) 4b moderate suspicion for malignancy(>10% to <50%) 4c high suspicion for malignancy (>50% to <95%
5	Highly suggestive of malignancy	Tissue diagnosis	>95%
6	Proven malignancy by biopsy	Surgical excision when clinical appropriate	

٥٣ شارع ١٠ بالمعادى ت & ف : ٢٣٥٨٧٣٢٢ - ٢٣٥٨٥٨١٩ - ٠١١٠٠١٥١٣٨٠ - ٠١٠٢٨٤٨٧٦٩٥

مركز اورام دار السلام
قسم المعمل

Pathology Report

Name: ناديه محمد فؤاد

Age: 45years

Clinical data: Left breast suspicious lesion.

Specimen Type: FNAc from left axillary lymph node.

Date : 15/5/2018 **File Number: 728/2018**

Gross Description:

2 mls of reddish fluid received in a syringe for cytology.

Microscopic Examination:

The examined smear showed:

-Moderate cellular smear showing clusters of malignant epithelium cells exhibiting high N/C ratio . The background is haemorrhagic with a few mature lymphocyts. Complete lymph node excision biopsy is mandatory for confirmation and for IPT (ER,PR,Her2)

Diagnosis:

FNAc from left axillary lymph node

Positive for malignant cells for confirmatory complete lymph node excision biopsy .

PROF. Dr. Mona Sakr .

د. منى صقر

Dr. Inas Mohamed Ali.

مركز أورام دار السلام
قسم المعمل

مركز أورام دار السلام

Pathology Report

Name : ناديه محمد فؤاد 7-6-2018.

Age: 44 years

Clinical data: Left breast mass

Specimen Type: Breast tissue specimen and axillary lymph nodes

Date : 2-7-2018 File Number: 728

Gross Description:

Two specimens were received designated as 1&2

1 – A piece of mammary tissue 7x7 cm with partial skin covering 3x1 cm . Serial sections revealed a firm grayish white mass 2.8x 2 cm located 1 cm under the skin and 1.5 cm away from deep margin

2 - Fibrofatty tissue piece entangling multiple nodules up to 1x1 cm

Microscopic Examination:

Sections examined revealed a case of invasive duct carcinoma G 2 with desmoplastic stroma and mild lymphocytic infiltrate. Peritumeral lymphovascular invasion was present . Associated DCIS (Comedo,cribriform , clining) low to intermediate grade , constituting about 10 % of tumour area the extending margin were free of tumour extension . Axillary lymph nodes showed malignant deposits in 6 out of 11 nodes (6/11) with capsular invasion

Diagnosis:

Left breast mass

Invasive Duct Carcinoma G 2

T2N2Mx

All resection margins were free of tumour extension

Axillary lymph nodes status (6/11)

ER , PR , Her 2 are available on request

PROF. Dr.Mona Sakr

د. منى صقر

Dr. Inas Mohamed Ali.